

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 37.5 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 2 Gynecology Ward	Room/Bed: 1 B 2 Gynecology Ward/ 1 B 2 Bed 6	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 27-Mar-2023 14:06	D.O.D. : 03-Apr-2023	D.O. Print Report: 08-Feb-2026 15:27

FINAL DIAGNOSIS:

34 YR /P2L2/PREVIOUS 2 LSCS/NO COMORBIDITIES ADMITTED FOR TUBAL TUBAL RECANULATION/SCT

OPERATION PERFORMED:

FISH MOUTH NEO-SALPINGOSTOMY On 30-Mar-2023

SURGEONS: Dr Sarita Agrawal

ASSISTANT SURGEONS/SENIOR RESIDENTS: Dr Vijayalakshmi, Dr Prahasnita

TECHNICIANS/NURSES: Seema

OPERATIVE FINDINGS: PATIENT MADE TO LIE IN SUPINE POSITION AFTER AND URINARY CATHETERISATION. UNDER ALL ASEPTIC PRECAUTIONS, PARTS PAINTED AND DRAPPED. LOW MIDLINE VERTICAL INCISION GIVEN OF 5cm SIZE. ABDOMEN OPENED IN LAYERS. UTERUS IDENTIFIED OF SIZE 6 WEEK, WITH B/L NORMAL OVARIES. BOTH SIDE FIMBRIAL END ADHERENT TO OVARIES. INTRA UTERINE METHYLENE BLUE DYE INJECTED THROUGH FOLEYS. DYE SEEN UPTO PROXIMAL END OF TUBAL LIGATION SITE. MICROSLAB INCISION GIVEN OVER THE TUBAL END. LUMEN IDENTIFIED WITH INFANT FEEDING TUBE. 3 STITCHES TAKEN TO EVERT SEROSA AND MUSCULAR LAYER OF THE TUBE TO CREATE FISH MOUTH END OPENING BY PROLENE 5 SUTURE AT 3, 9 AND 12 O'CLOCK. SAME PROCEDURE DONE ON THE OTHER SIDE OF TUBE. CONTINUED IRRIGATION WAS DONE THROUGHOUT THE PROCEDURE. TUBAL PATENCY WAS ASSURED WITH EVIDENCES OF SPILLAGE ON BOTH TUBAL END AFTER INJECTING METHYLENE BLUE DYE. ADEQUATE HEMOSTASIS WAS ACHIEVED, SALINE WASH GIVEN AND HYDROCORTISONE IRRIGATED INTO ABDOMINAL CAVITY OVER B/L SITE. ABDOMEN CLOSED IN LAYERS. SKIN WAS CLOSED IN VERTICAL MATTRESS SUTURE FASHION WITH ETHYLON 2.0. DRESSING DONE.

CHIEF COMPLAINTS:

INABILITY TO CONCEIVE

HISTORY OF PRESENT ILLNESS: 34 YR /P2L2/PREVIOUS 2 LSCS/NO COMORBIDITIES ADMITTED FOR TUBAL TUBAL RECANULATION/SCT.. HER VITALS WERE STABLE AT THE TIME ADMISSION.

GENERAL EXAMINATION:

GC - FAIR

PR- 88/MIN, RR-20/MIN, BP-138/82 MMHG

TEMP-AFEBRILE

PALLOR/ICTERUS/CYANOSIS/CLUBBING/EDEMA ABSENT

SYSTEMIC EXAMINATION:

CVS/CNS/RS-WNL

PA: SOFT, NON TENDER

PS: CERVIX CENTRAL, OS LSIT LIKE, SMOOTH REGULAR SURFACE WITH REGULAR MARGINS AND NO WHITE DISCHARGE.

PV: CERVIX CENTRAL, OS SLIT LIKE SMOOTH,MOBILE NON TENDER, NORMAL IN NATURE AND CONSISTENCY.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
31-Mar-2023 10:05	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.00	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	0.54	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.09	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	10.66	2 - 7 x10 ³ /μL
		Basophil	0.1	00 - 01 %
		Eosinophils	0.0	01 - 06 %
		HCT	34.8	36 - 46 %
		Hemoglobin	10.6	12 - 15 g/dl
		Lymphocytes	4.8	20 - 40 %
		MCH	22.6	27 - 32 pg
		MCHC	30.5	31.5 - 34.5 g/dL
		MCV	74.4	83 - 101 fL
		Monocytes	0.8	2 - 10 %
		Neutrophils	94.3	40 - 80 %
		Platelet Count	234	150 - 410 x10 ³ /μL
		RBC	4.68	3.8 - 4.8 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	15.6	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	11.30	4 - 10 x10 ³ /μL
27-Mar-2023 20:11	FASTING GLUCOSE	Fasting Blood Sugar	82	70 - 110 mg/dl
	PP GLUCOSE	Glucose P P	71	< 140 mg/dl
27-Mar-2023 19:38	URINE ROUTINE AND MICROSCOPIC EXAMINATION (R/M)	Appearance	CLEAR	-
		Bilirubin	NOT DETECTED	-
		Casts	NOT DETECTED	-
		Colour	PALE YELLOW	-
		Comments	Refer Reports*	-
		Crystals	NOT DETECTED	-
		Epithelial Cells	2-5	0 - 5 / hpf
		Glucose	NOT DETECTED	-
		Ketones	NOT DETECTED	-



27-Mar-2023 17:07	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	Nitrite	NEGATIVE	-
		Others	NOT DETECTED	-
		Protein	NOT DETECTED	-
		Pus cells	NOT DETECTED	-
		RBC	NOT DETECTED	-
		Reaction pH	7.0	5 - 7 .
		Specific Gravity	1.000	1.003 - 1.035 .
		Urobilinogen	NORMAL	-
		APTT HEADER	23.3	23.2 - 32.4 sec
	COMPLETE BLOOD COUNT (CBC)	APTT NOTE	Refer Reports*	-
		clinical details	--	-
		Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10^3 /μL
		Absolute Eosinophil Count	0.04	0.02 - 0.5 x10^3 /μL
		Absolute Lymphocyte Count	1.05	1 - 3 x10^3 /μL
		Absolute Monocyte Count	0.24	0.2 - 1.0 x10^3 /μL
		Absolute Neutrophil Count	4.18	2 - 7 x10^3 /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	0.7	01 - 06 %
		HCT	34.1	36 - 46 %
		Hemoglobin	10.6	12 - 15 g/dl
		Lymphocytes	19.0	20 - 40 %
		MCH	23.2	27 - 32 pg
		MCHC	31.1	31.5 - 34.5 g/dL
		MCV	74.6	83 - 101 fL
		Monocytes	4.3	2 - 10 %
		Neutrophils	75.8	40 - 80 %
		Platelet Count	251	150 - 410 x10^3 /μL
		RBC	4.57	3.8 - 4.8 x10^6 /μL
		Red Cell Distribution Width (RDW CV)	15.7	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	5.52	4 - 10 x10^3 /μL	
	PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.0	-	
	PT INR Header	10.1	09.6 - 12.2 sec	



PT INR Note

Refer Reports*

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***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: BLOOD GRP-A POSITIVE

T3-128.4(18/8/2022)

T4-7.94(18/8/2022)

TSH-1.52(18/8/2022)

USG OF ABDOMEN-NO ABNORMALITY SEEN(29/8/2022)

HOSPITAL COURSE: 34 YR /P2L2/PREVIOUS 2 LSCS/NO COMORBIDITIES ADMITTED FOR TUBAL TUBAL RECANULATION/SCT FOR WHICH WAS SHE WAS ADMITTED ON 27.3.2023.HER VITALS WERE STABLE DURING ADMISSION.ALL RELEVANT INVESTIGATIONS SENT. PAC CLEARANCE TAKEN. SHE UNDERWENT FISH MOUTH NEO SALPINGOSTOMY ON 30.3.2023.THERE WERE NO POST OP COMPLICATIONS. PATIENT PASSED STOOL AND URINE. POD 3 CHECK DRESSING WAS DONE.PATIENT IS HEALTHY AND READY FOR DISCHARGE.

CONDITION AT DISCHARGE: GENERAL EXAMINATION: PR:80/MIN, BP: 110/68 MMHG, RR: 18/MIN, TEMP: AFEBRILE

PALLOR/ICTERUS/CYANOSIS/CLUBBING/LYMPHADENOPATHY: ABSENT; EDEMA: ABSENT

SYSTEMIC EXAMINATION: CNS/CVS/RS-WNL

P/A SOFT NONTENDER, STITCH SITE HEALTHY

L/E NO ACTIVE BLEEDING PV

ADVICE ON DISCHARGE:

FOR 4 HIGH PROTEIN DIET.

TAB CEFIXIME 200MG BD DAYS

TAB METRO 400MG TDS FOR 4 DAYS

TAB PANT 40 MG OD FOR 4 DAYS

TAB PCM 500MG TDS FOR 4 DAYS F/B SOS IF PAIN

TAB ONDEN 5 MG SOS

TAB CALCIUM VITAMIN D3 BD X 3 MONTHS

TAB TRAMADOL 500mg SOS

TAB IRON 1 TAB PO OD FOR 3 MONTHS

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 08-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : FOLLOW UP IN GYNAEC OPD ON 8/04/23 FOR SUTURE REMOVAL WITH HPE REPORT OR SOS IN CASE OF SEVERE PAINABDOMEN, BLEEDING PV, DISCHARGE FROM STITCH SITE OR FEVER

Summary Prepared By

Summary Approved By

anonymous

anonymous